AdventHealth

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Department of Health and Human Services
Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
Attention: Request for Information: HHS Health Sector AI RFI
Mary E. Switzer Building
Mail Stop: 7033A
330 C Street SW
Washington, DC 20201

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care [RIN 0955-AA13]

On behalf of AdventHealth, we appreciate the opportunity to respond to the Department of Health and Human Services' Request for Information (RFI) on *Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care*. AdventHealth is a faith-based, not-for-profit health system with more than 50 hospital campuses and hundreds of care sites across nine states, serving over five million patients annually. Our system includes large academic medical centers, community hospitals, and rural facilities, providing a broad perspective on the opportunities and challenges associated with the responsible use of artificial intelligence (AI) in clinical care.

AdventHealth supports HHS's efforts to promote the safe and effective adoption of AI technologies that enhance patient care, strengthen the health care workforce, and improve healthcare outcomes.

Barriers to Private-Sector Innovation and Adoption of AI in Clinical Care

The most significant barriers to private-sector innovation and adoption of AI in clinical care include regulatory uncertainty, unresolved liability exposure, fragmented data environments, and workforce capacity constraints, particularly for small, rural, and safety-net providers.

Despite rapid advances in AI capabilities, the absence of clear, nationally consistent expectations regarding AI governance, validation, and accountability creates

hesitation among health systems when determining which tools are appropriate for clinical deployment. This uncertainty is especially pronounced for AI tools that support clinical workflows but do not clearly fall within existing medical device regulatory frameworks. Health systems are often left to independently assess safety, reliability, and legal risk, slowing adoption and disadvantaging providers with fewer internal resources.

Liability concerns further compound these challenges. AI systems operate at a speed and scale that can amplify even low error rates, raising questions about accountability when AI-supported recommendations influence clinical decision-making. **Without clear guidance or safe harbor pathways, providers may feel compelled to duplicate AI outputs through manual review, undermining the efficiency and workforce benefits AI is intended to deliver.**

AI as a Workforce Sustainability Tool and the Importance of Administrative Relief

From AdventHealth's experience, AI tools have demonstrated the greatest and most immediate value when deployed as *time multipliers,* particularly in administrative and documentation-related workflows. AI-enabled ambient documentation, chart summarization, and workflow automation have the potential to return meaningful time to clinicians, improving both efficiency and professional satisfaction. For example, AdventHealth's use of certain AI-enabled ambient voice technology, has resulted in approximately 96 hours saved per employee a year.

Peer-reviewed evidence increasingly supports this experience. Recent multi-site studies published in *JAMA Network Open* and related journals have found that the use of ambient AI documentation tools is associated with measurable reductions in administrative burden and self-reported clinician burnout, as well as improvements in perceived well-being. Randomized controlled trials evaluating large language model–enabled ambient scribes have similarly demonstrated reductions in documentation time and task load, with downstream benefits for clinician work exhaustion. In multi-site evaluations, ambient AI documentation tools were associated with reductions in note-writing time on the order of 20–30 percent per encounter and statistically significant decreases in burnout and work exhaustion scores. Randomized controlled data similarly demonstrate measurable reductions in cognitive workload and documentation burden compared to usual workflows, particularly among high-utilization clinicians. While continued evaluation is warranted, these findings underscore AI's potential role in addressing one of the most pressing challenges facing the health care system – clinician burnout.

These benefits are most pronounced when AI tools are integrated into clinical workflows in a manner that is trusted, validated, and supported by clear governance expectations. AdventHealth deploys clinical imaging AI platforms, which flags critical images for clinicians in real time, saving our clinicians about 45 minutes when making a diagnosis. The technology acts as a layer of critical review by scanning medical images to detect acute abnormalities like hemorrhages or fractures, helping clinicians to prioritize those images. Our physician in turn can appropriately triage and prioritize treatment and improve clinical outcomes.

Regulatory, Payment, and Programmatic Opportunities to Incentivize Effective AI Use

AdventHealth encourages HHS to prioritize the following areas to accelerate responsible AI adoption:

1. Exceptions and Safe Harbor Frameworks

HHS should provide exceptions and safe harbors pursuant to the Stark Law and the Anti-Kickback Statute for health systems that deploy or donate AI tools meeting recognized national standards for validation, documentation, and governance to its clinical partners. Such protection would reduce duplicative internal validation efforts and provide greater confidence for providers and provider cross organizational validation. Some AI tools, such as AI-driven diagnostic imaging, predictive patient monitoring, or generative AI administrative aids, do not clearly fit into the current Electronic Health Records or cybersecurity exceptions and would benefit from a similar protection.[1]

2. National AI Governance and Validation Standards

Clear federal guidance outlining accepted AI risk-management and validation frameworks would reduce regulatory fragmentation across states and set a federal

[1] Stark and AKS Final Rules Will Facilitate Donations of EHR and Cybersecurity Technology and Services

floor. Establishing consistent expectations, developed with input from both clinicians and AI developers, would promote interoperability, scalability, and equity in AI adoption.

3. Payment Policy Alignment

Currently, Medicare payments have been limited to very few AI-related Common Procedural Terminology (CPT) codes. **If HHS wants to accelerate the use of AI, payment should be better aligned with AI-enabled efficiencies.** For example, allowing appropriate reimbursement pathways for AI-supported clinical and administrative functions, including documentation and interpretation support, could help address workforce shortages while maintaining patient safety.

Non-Medical Device AI and the Need for Coordinated Federal Guidance

For AI tools not regulated as medical devices, health systems face unresolved challenges related to liability, data governance, privacy, and accountability. **HHS can play a constructive role by aligning expectations across ASTP/ONC, FDA, OCR, and CMS, clarifying oversight responsibilities and ensuring consistent guidance on documentation, transparency, and risk management.**

Additionally, HHS should remain attentive to the expanding ecosystem of consumer-facing AI tools that are not subject to HIPAA protections. Clear distinctions between clinically validated, provider-supported AI and general consumer AI are essential to maintaining patient trust and informed decision-making.

Evaluation, Benchmarking, and Post-Deployment Monitoring

Robust AI evaluation (both pre- and post-deployment) is essential to sustaining trust and accelerating adoption. **AdventHealth encourages HHS to explore using a framework, similar to the one used by Patient Safety Organizations under the *Patient Safety and Quality Improvement Act*, where users of AI in clinical care could safely report issues with AI technology that anyone can access and learn from.** This would create a protected environment for AI monitoring, evaluation, and continuous improvement.

Federal support for applied research, benchmarking, and post-deployment monitoring through grants, cooperative agreements, or pilot programs would also help health providers – of all sizes and resources – to deploy AI technology.

Conclusion

AdventHealth appreciates HHS's leadership in seeking stakeholder input on the future of AI in clinical care. AI holds significant promise to improve patient outcomes, enhance operational efficiency, and support a sustainable health care workforce. Realizing this potential will require regulatory clarity, aligned payment policies, and evidence-based governance frameworks that enable responsible adoption without imposing unnecessary barriers, particularly for smaller and rural providers.

We welcome continued engagement with HHS on these issues and appreciate the opportunity to contribute our perspective.

If you have any questions or would like further information, please do not hesitate to contact me or Susana Molina Ramos, Director of Public Policy at Susana.MolinaRamos@AdventHealth.com.

Sincerely,

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AdventHealth